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Increasing Comfort and Support in the New Graduate Nurse Through Focused Curriculum Delivery

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ABSTRACT

Introduction: The literature underscores a critical gap in preparing early career nurses (ECNs) for leadership roles such as shift charge nurse. Many nurses expressed surprise or uncertainty at being asked to lead, particularly early in their careers. Despite their varied clinical experience, nurses commonly relied on observation and mentoring to build the skills needed for effective leadership. The transition was described as both challenging and transformative, revealing the need for structured support. The goal is to evaluate the impact of a content-focused curriculum on the comfort and support levels of nurse residency program participants assuming charge nurse or preceptor roles.

Methods/Implementation Strategy: The proposed study will be a descriptive, quantitative cross-sectional study to evaluate the impact of the content-focused curriculum on nurse residents’ self-reported comfort, support and leadership readiness in assuming charge nurse or preceptor roles. The study will compare responses across multiple study will compare responses across multiple residency cohorts, including cohorts that received the curriculum (cohort 27 to 29) versus cohorts that did not receive (cohort 24 to 26) the content-focused curriculum, to assess differences in perceived support, comfort and readiness in the charge or preceptor roles.

Next Steps: Future work- evaluate data

BACKGROUND

The literature underscores a critical gap in preparing early career nurses (ECNs) for leadership roles such as shift charge nurse. Ting et al. (2024) conducted a qualitative study in Ontario where newly practicing RNs described their experiences transitioning into the charge nurse role. Participants reported being assigned these roles without adequate understanding or preparation, often relying on informal learning methods like shadowing experienced colleagues. The findings highlighted a lack of organizational support structures, with minimal professional development activities explicitly targeting the charge nurse role. These deficits were linked to concerns about patient and staff safety and emphasized the need for clear role expectations and leadership development opportunities.

Similarly, Doherty et al. (2021) explored the transition experience from bedside nurse to charge nurse and found that nurses often assumed leadership responsibilities with little formal preparation or desire. Their study identified three phases of role development: initial interest and recruitment into the role, orientation and learning, and eventual adaptation. Many nurses expressed surprise or uncertainty at being asked to lead, particularly early in their careers. Despite their varied clinical experience, nurses commonly relied on observation and mentoring to build the skills needed for effective leadership. The transition was described as both challenging and transformative, revealing the need for structured support.

PURPOSE

To evaluate the impact of a content-focused curriculum on the comfort and support levels of nurse residency program participants assuming charge nurse or preceptor roles.

POPULATION AND SAMPLE

The study population consists of nurse residents at Henry Ford Macomb across cohorts 24–29. Cohorts are categorized into control and intervention groups based on receiving of content-focused leadership curriculum. Specifically, cohorts 24 to 26 did not receive the curriculum are in the control group, and cohorts 27 to 29 received the curriculum are in the intervention group. Residency cohort timelines are:

- Cohort 24 dates from 9/9/23-8/7/24
- Cohort 25 cohort dates from 12/1/23-11/21/24
- Cohort 26 dates from 2/26/24-1/14/25
- Cohort 27 dates from 5/23/24-4/16/25
- Cohort 28 dates 8/29/24- 7/16/25
- Cohort 29 dates 10/25/24- TBD

METHODS/IMPLEMENTATION

The proposed study will be a descriptive, quantitative cross-sectional study to evaluate the impact of the content-focused curriculum on nurse residents’ self-reported comfort, support and leadership readiness in assuming charge nurse or preceptor roles. Data will be collected using selected questions from the Vizient/Casey-Fink Survey ©; a validated tool widely used to assess the experience and transition readiness of newly licensed nurses. Most questions are based on a 4-point Likert scale (ranging from 1 = strongly disagree to 4 = strongly agree). The Vizient/AACN Nurse Residency Program™ (NRP) provides guidance to newly licensed graduate nurses who are transitioning to their first qualified role. The program is partnered with the American Association of Colleges of Nursing (AACN) and its curriculum is based on the following domains: development of the professional nurse, foundations of nursing quality and safety, interprofessional practice, knowledge for nursing practice, leadership and systems-based practice, person centered care and scholarship for nursing practice (Vizient, 2025). The Casey Fink Survey© is utilized to evaluate transition-to-practice program outcomes, measure experiences of graduate nurses, assess readiness for practice, and identify factors that affect nurse retention (CaseyFinksurveys.com, n.d.).

The study will compare responses across multiple study will compare responses across multiple residency cohorts, including cohorts that received the curriculum (cohort 27 to 29) versus cohorts that did not receive (cohort 24 to 26) the content-focused curriculum, to assess differences in perceived support, comfort and readiness in the charge or preceptor roles.

DATA ANALYSIS PLAN

Descriptive statistics (means and standard deviations) will be used to summarize participant responses to selected items from the Vizient Casey Fink Survey©. The primary outcomes include nurse residents’ self-reported comfort, perceived support, and leadership readiness in assuming a charge nurse or preceptor role. Each outcome will be treated as a continuous variable based on 4-point Likert scale responses. Comparison between the control group (cohort 24-26) and intervention group (cohort 27-29) will be conducted using independent-samples t-tests. Normality and variance assumptions will be assessed prior to conducting the t-tests.

The study will include nurse residents from 6 cohorts (24-29) at Henry Ford Macomb. Cohort 24 – 26 did not receive the curriculum as the control group includes 37 nurses, while cohort 27 – 29 received the curriculum as the intervention group includes 56 nurses, yielding a total sample size of 93 participants. A two-group t-test with a 5% two-sided significance level will have 80% power to detect a difference in means of 0.3, assuming that the common standard deviation is 0.5, when the sample sizes in the two groups are 37 and 56, respectively (a total sample size of 93). The assumption of a 0.5 standard deviation is based on findings from previously published studies using the Vizient Casey Fink Survey©, which reported standard deviations between 0.41 and 0.60 across subscales in perceived support and leadership confidence (Baker & Alghamdi, 2020).

NEXT STEPS

Since submitting for Research Conference, IRB has been accepted.

Future work- evaluate data

REFERENCES

